pre-liver poetic transplant manual;
dedication ::

donors
patients
families //

++ WARNING ++
someone may offer you
money
to move up the list.
please call ==== ~~416-340-5163~~
— what they are suggesting is illegal —

[[[[[[[[[[[[[[[[[[[[[[[[[[[[[[[[*Human Tissue Gift Act*]]]]]]]]]]]]]]]]]]]]]]]]]]]]]]]]

we cannot accept gifts.
you are the gift.

~~introduction:~~
dear soul fused with a body,
holy,
so perfect incarnation
you are being considered for solid organ transplantation.
welcome.
first you or your caregiver must
obtain drug coverage // lots
find a dwelling for recovery ;; 2-3 months
teamwork || let's heal together
to and from the hospital :: back and forth, back and forth

~~consent:~~
consent to the awareness you and your donor are holy.
this has been explained to you. Keep it with you. Carry it with you.
by signing ;;; you acknowledge this.

XXXXXXXXX
sign here — *patient*
— caregiver
— bare witnessing :::::

~~UNIVERSITY HEALTH network~~ header goes here

~~{{{{{{{}~~Alcohol Abstinence Agreement~~{}}}}}}}~~

it is	important	, that you must know	||	your liver disease	has
had	been caused	by	[]	or	has
been	getting worse	because	you	were	drinking
alcohol					
in the	past.				
I touch	your chest	:: sensing	*why*	*the pain*	of
hungry	ghosts.	with a long	\\	neck;;	only
enough	space for	one sip	at	a	time,
with a	big	belly	that can		never
be fed	enough.	So we	must be		kind
not to	feed this	hungry ghost	because {}		it
will	never be	enough.	It will ‘””		just
want	more;;	never	enough.		So
we	must learn	to not give it	what	it	wants,,,
we	must learn	to give it	what	it	needs///
an	embrace.	and maybe	it will		sleep
for	an hour,	a day	a decade		and, then
it will	wake up.	but you do not	feed	it	you
embrace	it.	give it	what	it	needs.

///yes?\\\ or ///no?\\\

I am sorry for this rule: **to be considered for a liver are those patients who have ~~stopped drinking~~ for at least <u>6 months</u> and agree to completely *abstain* from drinking alcohol before and after liver transplantation.**

To be considered: liver transplantation

<you may be required to>
receiving counselling to remain abstinent for
alcohol
provide evidence of this
agree to <u>random</u> blood
urine samples
to test for alcohol
in your body
tangled in your mind
hiding the pain

If you cannot commit to these requirements, you will not be listed for liver transplantation,

UHN Multi-Organ Transplant Program
Alcohol Abstinence Agreement {header goes here}

Here is the agreement:
I, XXXXXa body fused with a soul XXXXX , understand that the practice of the ~~Toronto General Hospital Liver Transplant Program~~ **FORBIDS** me from drinking alcohol before and after liver transplantation.
and it goes on,
I also understand that I will only be considered a candidate for transplant *IF*
I stop drinking
and remain abstinent
from alcohol.

~~permission for samples to test for alcohol use:~~
you may take my blood and urine and, yes, a positive test means I used alcohol and I *will be removed* from this gift.

commitment to stop:
I know the consequences of my pain

XXXXXXXXX	**********	|||||||||||||||||||||||||||
patient	signature	date
XXXXXXXXX	**********	|||||||||||||||||||||||||||
caregiver	signature	date
XXXXXXXXX	**********	|||||||||||||||||||||||||||
bare witness	signature	date

did I translate this well enough?

will you listen to my voice reading this to your soul?
do you understand you are holy?

WELCOME!

(\ we are your liver transplant team /)

We are a large transplant centre
We weave and knot 140 livers a year ;; with great success
We have a long history of pioneering efforts; you are in good hands

We offer patients with end stage liver failure hope
We do this with living donor and deceased donor gifts
Live donor liver transplantation can have shorter wait times and improved survival
but both living and deceased donor liver transplants are good for you.

please share this manual with friends and family because this is a team effort.

++++

&&&&&&&&&& OUR PHILOSOPHY OF CARE &&&&&&&&&&

++++

< our remarkable donors and families make this possible, *what a gift you are* >
< we believe that respect, dignity, integrity, and empathy enhance carry and support ::
expect our love >
< we work in partnership with you and your family and the community to
promote care and quality of life, *the whole way through* >
< transplantation is a very specialized area in health care //
to succeed we need the knowledge and skill of our whole team
including you >
< each member of your team is a dedicated professional making valuable contributions
to your plan of carry. Always striving to improve >
< we see you, radiant jewel on the great net of being. We acknowledge you are unique
with your own needs, goals, and abilities, but also acknowledging you inter-be with your body,
mind, family, community, transplant team, donor's body, the soil, fire, gift of breath, sun, moon,
and cosmos >
< you | we are a collaboration >
< we hope to help you make informed decisions about your health carry >
< we base this on your needs, the resources scattered around, and our collective
ethical principles >
< let us care for you, *carry you* in a safe environment that supports and embraces you >

```
<!doctype html>
<html>
<h1>
                                                    ++++
############## OUR EXPECTATIONS OF YOU ##############
                                                    ++++
</h1>

<body>
<a href=”your soul”>you have the responsibility to</a>
<p> work in <em>partnership</em> to ensure the best possible </p>
<ul>
<li>treatment</li>
<li>rehabilitation</li>
<li>discharge planning</li>
<li>follow-up care</li>
<br>
<p>provide information and concerns to all members of your team</p>
<p>please let us know what you do not know</p>
<p>respect the reason everything happens, we will not wound you</p>
<p>treat staff, other patients, other families, and yourself in a considerate, courteous,
confidential, and cooperative manner.</p>
<p>~~Toronto GeneralHospital’s~~ role is also to teach and research and partner with those
learning</p>
<p><em>we recommend quitting smoking</em></p>
</body>
```

.css

```
body {
  background-color: cyan;
}

p {
  color: blue;
}
```



;; your transplant team ;;

You are about to go on a journey, a transplant journey, we we are here to teach you how to care for your organ
care for your health and soul
care and support you through this journey.

Your transplant team
<<<

###**doctors**###
###**social workers**###
###**nurse practitioners**____
###**physio // occupational therapists**###
___**dietitians**___
@@@**others**@@@
of course

XXXX**nurses**XXXX
xxxx**transplant coordinators**xxxx
XXXX**spiritual care workers**xxx
xxxx**psychiatrists**XXXX
XXXX**pharmacists**XXXX
&&&and& & &
you, *radiant jewel* >>>

on this journey, there may be other wayfinding figures

~~hepatologist~~
doctor: highly skilled in the diagnosis and treatment of liver disease.
they care for you before and after, behind and ahead

~~liver transplant surgeon~~
doctor: patient evaluation and selection,,,
removes, then weaves and knots a gift within you
recovery

~~psychiatrist~~
doctor
maybe nurse
cope, illness
that which arises

~~transplant coordinator~~
nurse evaluation process
education
follow-up care, carry

~~medical secretary~~
administration relay concerns

~~social worker~~
understanding you
family, support

~~physiotherapist~~
collaboration to gain OPTIMAL
strength and flexibility

~~others~~
GP, liver specialist
still;; primary source of care, carry
don't forget to get care, regular
we || will all work together

++++++++++++++++++++why you need a liver transplant++++++++++++++++++++++++++++++

it has been suggested that you need a liver transplant. To understand, first we must understand how it works.
Liver: largest solid organ in your <body>
Liver: has many function to support life, the process of healing that which wishes to unwind::::

Liver: ^^^manifests special proteins to prevent bleeding
^^^filters blood :: fights infection
^^^manifests bile,, shatters fats
^^^strings together sugars, creates dwellings for them, then releases them like a quake
^^^embraces vitamins and minerals
^^^breaks into bits proteins in the food you eat
^^^sends cascading signals {hormones}

when disease damages your liver, it does not function properly, the pull of entropy, the pull to disorder begins, *the strings begin to unravel*.

there are many names given to processes that begin this unraveling :::::::

viral hepatitis: B or C	alcoholic hepatitis	N.A.S.H.
P.B.C.	P.S.C.	Alpha-1-antitrypsin deficiency
drug-induced hepatitis	Budd Chiari Syndrome	Congenital fibrotic disease
Cryptogenic Cirrhosis	hemochromatosis	hemochromatosis
polycystic liver disease	Wilson's disease	

in +++++++addition+++++++ to liver failure, liver cancer may develop as a result of many of these diseases xxx

sometimes, cases ;; sudden liver failure with ~~unknown~~ causes called: acute, or subacute fulminant liver failure

signs & symptoms of LD

a diseased liver cannot carry out{you}normal functions.

People with diseased or failing livers may experience:

{} ascites (fluid in the abdomen) /// Rick, you had his stomach drained with a needle and bag monthly. The last time I saw you was at the ~~**UofA Hospital**~~ having this done. It was April 2015, I was installing an exhibition on liver transplantation at the ~~McMullen gallery~~. You died in July the day after my birthday.

{} fatigue // I feel into my fatigue, then you slept for days before the unraveling finished

{} confusion /// if only you hadn't the pain of his mother abandoning, if only you didn't always have a beer in your hand, if only you went to your doctor for check ups, or at least when beer made you ill and your weight began to shed, *if only,*

{} change in sleep patterns // near the end, you woke up paralyzed and yellow, my aunt called my mom in terror.

{} itching /// scratching my arm, scratching to the marrow to imprint drawings of ~~me~~ or ~~you~~

{} easy bruising // can't quite stop the bleeding, Rick.

{} nausea |vomiting| // // a beer or two shouldn't make you vomit, *something was wrong*

{} muscle cramping // *why the pain of ... *

{} swollen ankles // lead feet holding you down [only for a while] from heaven

{} dark urine /// [remember: you may take my blood and urine and, yes, a positive test means I used alcohol and I *will be removed* from this gift.]]

{} fever and infections // you were a *supernova*

{} pain over the liver // watching you die, I felt like I was watching myself die.
i wanted to quit drinking while watching you unravel.
even now, quitting on December 20, 2019
all the nerves still tingle under my skin in may arms, and stomach writing this. Like I'm still feeling that unimaginable fate.
saying goodbye to ellie | mary;; like your sad goodbye to meagan | jessica

{} internal bleeding // *if only this damn liver would make those proteins*

{} jaundice (yellowing skin :: yellowing eyes) // I could not see you when you are yellow

{} spider viens (shattering blood vessels // unraveling

{} pale stools, black stools, fatty stools ////////////////////// ~~gone~~

please note: not everyone experiences all these symptoms. I did, just thinking of you, writing for you, uncle Rick

a liver is a marvelous organ. If damaged, if injured, if unraveled, it may be able to grow new cells, weaving and knotting itself back together, the string figures of your body bracing each other.

However ;;;

_____am i eligible for a liver transplant%_____

Indra spun a net that went up to infinity
that went across to infinity
and went forwards and backwards to infinity
and as the net overlapped, at the point of connection, ~~you~~ formed, a jewel on Indra's net
and in ~~you~~, in each jewel, every other jewel is reflected within it
in each jewel is every other jewel. ~~you~~ are the whole cosmos
the whole cosmos is in you
This is inter-being ;; connection

So when I say: each patient is assessed individually
I mean: *this jewel will be considered in relation to all the other jewels reflecting in it*
then I say: this jewel will be considered for its suitability for transplant.
knowing: that the whole cosmos, you, me, your donor, are all reflecting in each other.
there are no distinctions, only one.
This is why you are eligible.

Other requirements:

1. assessment reveals:
    a. you are unraveling and must be woven back together
    b. you can safely fall into the void of anesthesia and surgery.
2. you want to have a transplant ‘’’ you know what you are signing up for: living gift

our goal is to make this safe and successful, to carry you, [carry your]
both you and ~~you~~ (soul) are involved in care, carry decisions to understanding your treatments and checking you regularly to make sure your carry plan is working.

We need your and ~~your~~ commitment too; **<u>having a transplant will change your life.</u>**
significantly
before you make this choice,.,.,.,. you must be prepared to make many changes.
{{{follow your treatment plan}}}
this includes
:: being part of a your and ~~your~~ team
:: learning about your and ~~your~~ treatments
:: taking medications
:: attending clinical appointments
:: attending to your||~~your~~ body
:: make regular pilgrimages towards your~~<your>~~soul

|| Advantages & ~~Disadvantages~~ of Liver Transplantation ||

advantages ::

„::’ you have more energy to attend to your body and soul / to you loved ones / to your community,

„::’’ your diet is less restrictive and you can take pleasure in the vibrant fruits of incarnation,

„„::’’’ you do not have fluid restrictions and can forget your clothes and be baptized in the open ocean,

„„„::’’’’ your hemoglobin increases and can fully experience the giving origin in the breathe on your pilgrimage to the soul :

breathe in

breathe out

in

out in

out

„„„„::’’’’’ overall; your health is improved, the unraveling has been raveled, the string figures holding together a body with a soul have been mended, your potentials are limitless.

~~disadvantages:~~

~~„::’ you will need transplant~~ medication ~~(immunosuppressants and others)~~

for the rest of your life to prevent ~~rejection;~~

~~„„::’’ you will need care,~~ carry ~~for the rest of your life;~~

~~„„„::’’’ you may have side effects from your~~ medications, that may be better than the side effects of your illness

~~„„„„::’’’’ you may be at a greater risk for infection~~ <see next page>

~~„„„„„::’’’’’ you will be at great risk of cancer~~,

as you might see, there is an art to doing-illness. you will be an amazing artist.

infection risks with transplantation

we must consider the risk of infection when choosing to proceed with liver transplantation.
we hope this informs /| helps /\ carries you in this decision.

Infection is an ~~unavoidable~~ risk of transplantation.
about 1 in 3 ; the most common complication. but we must balance the risk / benefit

You are at a greater risk because of the ~~anti-rejection~~ drugs given after transplant that affect your immune system. *bacteria,*
viruses,
fungi,
or other organisms can cause infection.
Most can be treated, but some, well some are more difficult, and we must prepare for the chance they might cause disability or death. ~~no death / no fear~~

<u>We do have procedures to lower the risk and impact of infections by</u>

+ testing of the donor and of blood products // origin
+ we will give you anti-infective medications at the time of surgery
  sometimes after as well ;; and
+ we will monitor and test you, care and carry you

We are also learning more and more about this risk: growing. Learning more about the unknown.
more about: ~~wound infections ; abscesses ; pneumonia ; and urine infections~~
these are all potential complications of *any* surgery.

For you to be informed I need to list a few of these infections to your soul ; for your soul to consider your body, so perfect incarnation.

~~**multi-drug resistant bacteria**~~
no standard antibiotics

~~**hepatitis B or C**~~
we test before

~~**cytomegalovirus**~~
you might have some immunity

~~**epstein- barr virus**~~
same with this

~~**west-nile virus**~~
this is harder to detect but still, *relatively*, uncommon in ~~Ontario~~

~~**AIDS**~~
we test

~~**darkness covers the face of the deep or UNKNOWN INFECTIONS**~~
you have a weakened immune system now, but we are doing everything we can

~~non-alcoholic steatohepatitis~~
non-alcoholic fatty liver disease
can occur after *gift*.
OPTIMAL control of risk:
diabetes and body weight;;;
there is no drug therapy for
this. It is rare but possible
that this could lead, again,
to end-stage liver disease {}.

disease recurrence after transplantation
+++-

some diseases, resting in your body, carried by your body
may recur,
after this gift.

~~hep B~~
we have great medications for this to control hep b before transplant
and you will need to continue these after.
further you will receive injections of antibodies against hep b at regular intervals in
spacetime and spirit.
this is highly effective

~~hep c~~
if you are receiving a transplant for hep c related liver disease,
the gift is always re-infected.
there is no way to prevent this, ~~sorry~~
after transplantation hep c runs a more aggressive course, ~~sorry~~, that before
transplantation. Many will develop graft cirrhosis. ~~death~~
current antiviral therapy for hep c is less effective after transplantation,
has numerous ~~side effects~~ and can trigger rejection of this gift.
we only proceed with treatment if this progresses on your gift.
we will monitor this as best we can.

~~autoimmune liver disease~~
such as **~~autoimmune hepatitis (AIH)~~**
or **~~primary biliary cirrhosis (PBC)~~**
or **~~primary sclerosing cholangitis (PSC)~~**
may recur in gift;;; somewheres 10-30%&
typically controlled by medications;; but, soul:
this may lead to end-stage liver disease in your gift.

now the **~~hereditary liver diseases~~**
~~hemochromatosis~~ is not cured with a gift ::
and excess iron may build back up in this new liver;
best to monitor total body **iron** stores and to begin
again ~~phlebotomies~~ to keep things normal.

another
there is a defect that leads to excess copper IS cured
with liver transplantation and **~~wilson's disease~~** does not
recur in gift. HOWEVER, damage caused prior to gift
in other organs, such as the brain, is not reversible.

another
Good News is that your new liver does not carry the
genetic defect casing **~~alpha-1-antitrypsin deficiency~~**
and this does not recur in the gift. However [as above]
damage to other organs {such as the lungs} is not cured.

<<<**liver transplantation for ~~hepatocellular carcinoma~~**>>>

message from the ~~poet~~: I have never had a harder time being in my body and mind than when writing these last few poems. My body is crawling. My liver hurts {carry my heart};;; I feel like I need to see a doctor. I am so sorry you are going through this [c/o m. *I imagine your experience.*

~~introduction~~

we will now refer to it as ~~hepatoma~~.

is a malignancy {{ a metastatic instance}} that started in your liver: usually from

pre-existing liver disease

~~hcp b~~

~~c~~

~~alcohol~~

~~NASH~~ or

~~hemochromatosis~~

~~indication for transplantation~~

this gift is shown to be an effective treatment for small, early ~~hepatoma~~ that is dwelling only in your liver.

here are some criteria, useful to you or not, based on the size of the tumour:

+ 1 tumour ≤ 5 cm, or;;;
+ 3 tumours ≤ 3 cm.

a cancer-free survival is 70-80% following transplantation within this criteria. yet some larger may still be treated.

In **~~Toronto~~**, tumour that exceed this criteria {called the Milan criteria} may still be considered for a gift, if ::

= confined to liver
= without *invasion* into veins
= no constitutional symptoms
= and something I don't know how to describe,,, along the lines of[microscopically favourable]

for those of you listed with ~~hepatoma~~ that is >>>>>>>>>>>>>>> 2 cm in diameter and//or multifocal are eligible at slightly higher priority on the liver transplant waiting list.

~~treatment while waiting {{ AKA Bridge therapy }}~~

since liver transplantation is a HOPE // desire // VISION // and is NOT inevitable because of the organ *shortage* we recommend treating ~~hepatoma~~ as if your body is not listed for transplant, please take the best carry therapies available >>

strange to see you now without hospital bracelets or eyes, in the starry dinamo of night as you traverse the rivers across to the actual home of the ABYSS <<<<

the idea is to control tumour<s> during this waiting time *and*, specifically to prevent you from being taken off the list if it keeps spreading.

<<subheader: continued on the page>>

The bridging therapies used are:

— ablation ———— slipping a needle through your skin, then body, missing soul, and into the tumour and DESTROYING it by heating it with hi-frequency ~~radio-waves~~

— TACE ———————— Through the Artery Chemotherapy with Embolization – chemocial therapy sent through an artery directly into the tumour<s>/ your {body} will need 2-3 days in a hospital b ed.

— radiotherapy ———————— highly focused IRRADIATION that spares most of the liver;;; an {exclusion zone} …. 10 days out-patient,,, soul'

~~unfortunately~~, for many souls fused to a body, there is no effective bridge therapy avaialble because TOXIC____treatment and ::::risks:::: that come with advanced liver disease. ~~morcover~~, <soul,,, I am speaking directly to you '"" soul> despite all these efforts, up %25% d r o p off the waiting lists because that ~~goddamn~~ tumour keeps growing. ~~fuck~~

~~monitoring while waiting~~

we wi;; monitor your body<tumour<<S>>>> to ensure when it it your turn in the line, you will still be ""transplantable"" // To do this, your surgeon will watch CT and MRI scans of the chest and belly of *hungry ghosts* and run blood tests every 3 months, for you, if you are Milan criteria fitting.

which it it your turn in the line, you will still be ""transplantable"" // To do this, your surgeon will watch CT and MRI scans of the chest and belly of *hungry ghosts* and run blood tests every 3 months, for you, if you are Milan criteria fitting.

new or recurrent tumours will be blasted out of existence with Bridge therapy means. Yet, if it spread out the liver or wraps around your viens, like it did for you, ~~Rick~~,;;; you will no longer be eligible.

new or recurrent tumours will be bl
out of existence with Bridge therap
Yet, if it spread out the liver or wra
your viens, like it did for you, ~~Rick~~,

~~monitoring after liver transplant~~

because, immunosuppression is required after transplant to prevent rejection, this increases the risk of recurrence of your cancer,,, because ,,, we keep your ~~meds~~ low. There are risks,, whatever the %% i tell you. surveillance is our best hope. but if it does we will always work to {carry} you

==========the TRANSPLANT ASSESSMENT Process A++++++=========

~~<transplant coordinator>~~

We are going to assess if you are a transplant candidate.
Important: we need to ensure you can tolerate the {{physical}} and [[emotional]] weight of
During your assessment for liver transplantation, {} ~~<cardiologist>~~ this
you will meet many of the carry team {} experie
who will help you decide if this is the journey for you {} nce
~~<anesthesiologist>~~ both
Sometimes <> your assessment will uncover a problem {} the ope
that makes transplantation a poor option for <you> and ~~you~~ {} ration
;also; it may identify a problem that can be corrected {} and rec
before you become a carry/date {} overy

~~Your~~ assessment will include **extensive** medical test and interviews with members of the transplant team. We need to *ensure* you do not have any other conditions or health problems that would put you at too **high a risk** for a liver transplant.

[[[[[[[[[[[[[[[[[***To your soul: you must understand that a liver transplant is a very MAJOR OPERATION with significant risks:::::::: at the best of times***]]]

LIVING donor liver transplant_____

We have been carrying out live liver transplants since 2009. I met Fraser last month, he had one. These are just as successful as deceased gifts, yet they significantly reduce the risk of you getting sicker and sicker, falling and falling, portal open, ~~*abyss,*~~ waiting for someone's manifestation to unravel to gift you.~~no death no fear~~

Depending on your blood type, the reflection of you as a jewel, you might wait for years for a deceased organ gift, unraveling and unraveling in that time. A living gift can be arranged in months.

^^^^**benefits of living****

***** you will receive an exceptional quality gift, with excellent graft function
* short time waiting // unraveling // suffering
* it gives us time to plan for your best recovery before you become too ill to carry properly
* it lowers the chance you might die waiting for a liver
* success for you and the donor
* the donor has the opportunity to weave into you, embrace you, water the seeds of health, happiness, acceptance, and peace.

~~**####disadvantages#######**~~
& a higher chance of bile duct complications
& placing the otherwise healthy donor at risk (~~no death // no fear~~)

a living donor is a jewel that reflects all of you (friend family soul) with good health, with a similar blood type. Maybe you know someone that might want to learn more about living gifts, ask them to visit our website or give us a call

~~**living liver donor office @ 416-340-4800 ext 6581**~~
~~**www,uhn,ca/focus_of_care/mot**~~

_____—-living donor assessment—_____
we must evaluate the living donor by the carry team. *blood tests x-rays ultrasounds consultation with specialist* to provide information about what is to come.
(donors must be in excellent physical, emotional, and spiritual health) (no history of cancer or any active infection) (must have normal liver function) (must have suitable blood vessels and bile ducts to the liver) (the donor must have friend or family support before, during, after, and onwards)

~~(((((((~~principles guiding living gift~~)))))))~~

+ living donor must be neither too young nor too old, just right
+ living donation must be a voluntary gift / embrace / love / benevolence
+ donor safety is a priority
+ any new donor health issues must be carried for
+ it's the donors responsibility to speak and we will listen
+ ~~OHIP~~ pays for all the costs of this operation and the hospital care
+ there are other costs as well: time lost at work, travel, and so on that are not covered by us (but maybe the government)

matching donor and recipient

you two jewels must have a compatible blood type. Here is a helpful table

if your blood type is	**you can RECEIVE a liver from**	**you can donate to**
O	O	O,A,B,AB
A	A,O	A,AB
B	B,O	B,AB
AB	O,A,B,AB	AB

**********About 5% of live donor surgeries will be canceled days before giving. This is often due to additional findings or concerns that arise upon the team's final review of both jewels charts. I am sorry for this, you must be very frustrated, you must be very scared, maybe even angry. But we must be as safe as possible , ensure both of your continued safety. **********

=========considerations for living gift recipients========+

() you must be suitable for this, we will look carefully at your body and soul
() you agree to a living gift
() you agree we have read you all the information you need, you understand the successes and risks

~~<<<<~~risks associated with living gifts~~>>>>>>~~

survival rate between living and deceased gift after one your is the same: about 80-90% survive.
risks to ~~you~~: 1. there can be some complication weaving back your bile duct for living gift. 2. still rare, but your hepatic artery may clot.
risk to your ~~donor~~: all the risks of a major surgery

we believe living donation is the best choice for many.

our program has never had a death or long term complication for the donor.

the [costs] of transplant [medications]

(when it is all said and done, unraveled and raveled, alone and together, you must be ready to pay for meds)
(*sorry*, but this may be $4,000 per month)
(*please,* register to Trillium and pay the deductible, ~~or~~ you will need to pay by:
~~— drug benefit card~~
~~— VISA~~ or ~~Mastercard~~
~~— CASH~~)
(*again, [sorry],* the Transplant Unit will ***not*** provide medications to take to your home)
(the ~~**Toronto General Hospital**~~ ***does not*** have a drug assistance plan for you,
dear, transplant patient)

[drug] coverage — ~~private insurance~~XXXX

(who is the provider of your insurance?)
(what is the percentage of medication they cover?)
(Is payment automatic, or do you pay up front and get reimbursed?)
(Are there yearly maximums?)
(Are there lifetime maximums? ~~no death~~ ~~no fear~~)
(If you pay for your medications up front, *how do you plan to pay for your medications?* **YOU MUST HAVE A PAYMENT PLAN PRIOR TO THIS GIFT!!**)
(you should submit a list of your medications to your insurance company for pre-approval. We can get you this list)

*** REGARDLESS OF YOUR SITUATION AROUND DRUG (0\/3R@G3 THROUGH PRIVATE INSURANCE, YOU **MUST** HAVE ~~ONTARIO DRUG BENEFIT COVERAGE~~ — TRILLIUM, ~~SENIORS BENEFITS~~, OW / ODSP - - - - - - - - **IN PLACE** == ***

(if your transplant team needs to request special approval for medications that your private plan does not pay for, we can only do so with ODB in place ;; [therefore], we need to prepare early to prevent any delay in your gift carry)

{{header}} **<u>the TRILLIUM drug plan</u>**

many transplant drugs are expensive and unusual. ; . costing and costing
even the best plans can't quite cover it // the TRILLIUM drug plan || funded by
// is here for ~~Ontario~~ || the ~~Ontario~~
// residents to help, || government

ALL patients <u>must</u> be registered with the TRILLIUM drug program before being placed on the *transplant waiting list*

~~no fear~~, there is no cost to register with TRILLIUM.]]
[[the app only takes a few breaths
TRILLIUM does not interfere with private drug coverage]] [[if you have private coverage you can still use this program
[[it will help with costs that are not covered or only partially covered by private insurance]]
[[apply at pharmacies {} online {} or, through our social workers ;;

as a transplant patient, you <u>must</u> keep your registration alive and well with TRILLIUM ;;

{{renew every year before August, regardless if you are pre- or post-{{
what is this program?
it is here to help pay for part of your medication,
wait, how does this work?
well, you need to pay a yearly deductible that runs from Aug/1 to Jul/31. But join anytime,
and, you deductible is considered by the ## of people in your house and your income,
and, your deductible is digestible into quarters, no need to pay all at once.
ie. 2 adults // 1 child // $40K per year == $1.3K deductible. So 4 $325 payments and then TRILLIUM will take carry of the rest.
however, it only covers drugs on the ODB, but your doctor might prescribe others,
\\ doing our best.
who can apply?
one: you live in ON and have a valid OHIP
two: you are younger than 65.
what if I have more questions:?
contact TRILLIUM:

address	**phone**	**fax**	**email**
~~po box 337, st d etobicoke, onm9a4x3~~	~~TO: 416-642-3038 1-800-575-5386~~	~~416-642-3034~~	~~trillium@resolve.com~~

now,; onto :; the ., <u>financial information</u>

have a transplant can impact your finances. *we* must know and plan ahead.
your income may change; illness you may have new expenses.
every jewel is unique ~~(yet one)~~ use this info rmation
as a guide for opps for
financial help.

transplant patients may be eligible for such financial help:

< **<u>insurance</u>** :: **employment** and | **sickness benifits** >
you may have short | or you may have EI
or long term benefits | sickness benefits
contact your employer| contact your local EI office

< **<u>employment and retraining funding</u>** >
:: some patients may need to change their job ;
:: this program helps patients who have physical problems that prevent them from finding or keeping their job.
~~:: << *unfortunately*, there is a long waitlist >>~~
:: once accepted, you can receive financial assistance to be retrained.

< **<u>~~Canada~~ pension plan : disability</u>** >
If you are perminanently unable to work after your transplant <*soul fused with a body*> , and you paid into CPP, you can apply for CPP benefits. // you are approved based on the medical information they receive from your doctor. :: drugs are not included :: ;;

< **<u>~~Ontario~~ works</u>** >
:: if you have low income or little else you can apply for O W ;;
:: your total family income determines if you are eligible ;;

< **<u>~~Ontario~~ disability support program</u>** >
[if your doctor states you are permanently disabled and cannot return to work ; you may qualify ; for ODSP ;]
[drug benefits are included ;]

; and : **_____other financial considerations_____**

after your gift, these will be some tax tips — you will have other expenses: doctor and clinic visits. *Unfortunately*, out of pocket expenses. Here are a few below, and that will help you recover,,, from these expenses.

parking >> hospital parking is expensive >> the closer you park, the more you pay
look at further, nearby lots. or, consider public transit.

wheel trans >> if you live in ~~Toronto~~ and are physically disabled, you can apply for help.
this service is available to you and ~~you~~ if you are not mobile enough for regular transit.
to get a wheel trans #, you must have an interview with the ~~TTC~~.
call ~~416-393-4111~~.

housing >> sometimes you need to move to ~~Toronto~~ while you are waiting for transplant or after. Talk to your social worker about **a list of places to stay**

travel >> for those of you living in ~~northern Ontratio~~,
there is a travel grant to help,
you will need your doctors help,
social assistance // O W || ODSP \\ may help,
~~HOPE~~ Air ***may*** offer a reduced rate to help,
however ;; you need to book in advance

toll free: ~~1-877-346-HOPE~~
~~Toronto~~ area: ~~416-222-6335~~
~~www.hopeair.org~~///

other costs >> there are always other costs // special supplies // etc

{{{{{{{{{{some services require documentation for you to receive reduced rates}}}}}}}}}}

tax tips >> call ~~Revenue Canada~~ for info on deductions related to your transplant
hold onto your receipts! and, ;; you cannot claim re-imbursed expenses
a list of claims: long distance calls to the hospital ;; any diagnostics you paid for
;; payments to hospitals ;; drugs you paid for ;; premiums for private plans
;; ambulances ;; parking ;; out of ~~Canada~~ expenses

disability credit >> a doctor can help, with this. you include this on your tax return
take note: many transplant patients do not qualify for this credit

++++legal information for you, soul++++

&power of attorney&

yesterday I woke up,,, every since your situation, you might need to consider a power of attorney considering all in its right place, there are two PoA:

::::::: PoA personal care;;;;;;; PoA finances

(PoA for personal care)))))))

<<<<<<<<<<--------**after the assessment process**

after all the tests and tests and tests, the transplant team will meet an consider your results.
we hope you are good to go forward >>>>enter>>>>**waiting list** we will let you know by phone and letter>>

then>>>>... you will get a pager , care//carry this with you. give us several other contacts **we must be able to reach you>>**

we will>>>schedule>>>a meeting with the transplant surgeon
>>>bring>>someone with you to help you understand this raveling.
the surgeon will>>>>tell>>>>you about the successes and risks of liver transplantation:

>risk of death during surgery<
>primary non-function of the liver<don't worry I don't know what that means yet either
>hepatic artery ~~thrombosis~~<
>neurologic && other complications<
>severe infections<
>long, long stay in ICU<
>maybe, the need for re-operation< |I met a child that had this done,
|his living donor, his dad's liver failed
|he immediately got another one that was
|not as good. Not as healthy
|until he was 20 and he got another. a few tries
>the *commitment* of you and your family<
>maybe, you might enter some clinical trials<

you will also have a>>>>>>discussion>>>>>with the transplant hepatoligist.
it is important to>>>>discuss>>>the option of living donation.

l i v i n g d o n a t i o n h a s m a n y b e n e f i t s.

------->>>>>>>>

{{{{{{{{{{*waiting on the* list}}}}}}}}}}

keep, carry these ~~3~~ goals with you while waiting for transplantation ::

- **** maintain your body temple
- **** pay close attention to your body temple,
  notice and manage any new illnesses, unravelings
- **** take account and treat (if you are able) your signs of liver failure

at the ~~Pre-liver Transplant Clinic~~, you will have regular appointments while you wait and wait.

you must keep these appointments…. they are important opportunities to care, carry your health and sutability for transplantation.

at these body temple visits we may adjust your medications of order additional tests.
Assessment test will be redone every 6 to 12 months to ensure no new problems have emerged.

*** for those with tumors, it is especially important for these assessments completed to ensure your tumors have not become too large ***

~~blood work while waiting for liver transplantation~~

your body temple will need ongoing bloodwork while you wait and wait
please,,, have your blood work done at a local **~~life lab~~.**
these labs are familiar with our routine tests and work to complete your tests quickly.
they will not charge you for these test, while others might.

to find your nearest lab,,, call :: ~~1-877-849-3637~~ or ~~416-675-3637~~

LLLILLLLL your place on the list LLLLSTLLL

this waiting list, full of unique yet connected body temples, is created based on many things at any time there are ~~hundreds~~ of patients in ~~Ontario~~ waiting for a liver transplant.

each jewel on the great net of Being, each body temple,,, is given a status based on ::
OOO severity and type of illness OOO

it is important to notify your transplant coordinator if you are hospitalized while waiting.

your **status** might change as your health and urgency to stop the unraveling of one's body temple changes // this decision will be made by your transplant doctor.

gifts are allocated based on medical <u>need</u>. Not length of wait time.

{} If you have hep b or c, you can
{} receive a liver from a donor with
{} the same disease, but normal liver

---

neither the race nor sex of the donor nor you matter. Donors ;; however ;; must be:

* roughly the same height and weight as you *
* free of heart disease and ~~cancer *~~
* free of infection and chest trauma *

In ~~Ontario~~ livers are allocated according to:

* compatibility of blood types and liver size <body>
* <u>severity</u> and type of illness of you, dear soul

~~————~~maintainging contact~~————~~

you this *gift* becomes available,,, we must be able to get ahold of you
we must know how to contact you at all times. We need:

[[pager ##]]
[[home address//##]]
[work ##]]
[[cell ##]]
[[a friend or relative]]

keep your contact info//## updated

keep us informed if ::
[[any ##'s change]]
[[if you will be out of reach
where can we reach you?]]
[[are you admitted to another hospital]]

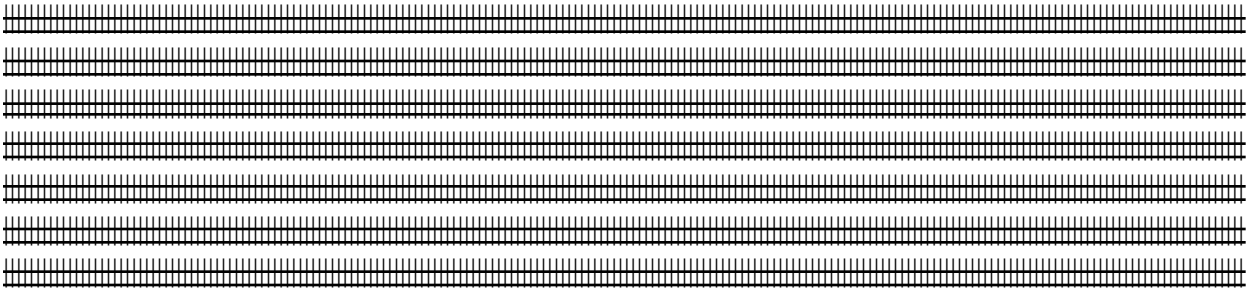
if <<for one reason or the next>> you are not prepared to come in <<at any time>>, you must **contact your ~~transplant~~ coordinator.**

!!!!!!!! if you are hospitalized while waiting ^^^^^^^^

if you are hospitalized while waiting for your *gift*, please contact your transplant coordinator.
it is up to the admitting doctor to update your transplant doctor with your status.
your ~~transplant~~ coordinator cannot change your status without your admitting doctor contacting the ~~program~~.

please find us at the ~~Toronto General~~ Hospital Locating services,,, request to speak to a transplant doctor

~~(416) 340-3155~~



____the ~~deceased donor~~ process____

if a living donor is not an option you must,,, wait for a ~~deceased~~ donor \\
in Canada, this gift is made possible by the kindness //and\\
generosity
of : the : donor's :
family;; who
consent to donate a
loved one's organs and
tissues,,, at
the time of *their*
~~death<<~~
the waitlist you will be *joining* is made of people throughout ~~Ontario~~
waiting, praying, taking hollow breathes
for a liver.

~~**The Trillium Gift of Life Network**~~ is the organization that takes care of this system in ~~Ontario~~.
they support donor families //and
organize the organ donation process //with
transplant centres.

*** once a *living gift* is found, the next of kin ||||| consent to the *gift*
*** this *living gift* is tested to make sure this *gift* is suitable for transplant ::
^^blood tests>>
^^ virus tests // HIV, Hep V && \\C \\ >>
^^ x-rays>>
<<scans and the *rest*^^
*** after testing, *gifts* are assigned to the appropriate patient :: waiting
*** soon, a transplant program will accept this *gift*,,, and the donor <radiate jewel> will
be brought into the operating room.
where,,, a specialized team of surgeons,,,
will *most carefully* remove the ogan for *gift.*

You wait time for a donor liver may be months,,, may be ~~years~~. Sorry <<soul>>,, this can be very stressful,, and/or,, discouraging for ~~you~~.

waiting, waiting, and ~~waiting~~ can be a very difficult time. There are resources to help you deal with this stress //^^jewel on Indra's net^^\\ … a place to start might be with the souls on your transplant team. //

header> helping you deal will *stress* <header_____>

<soul,, jewel> we do not know how long you will be on this waiting list .
sorry, <<soul, jewel> you/~~you~~ do not have control over this list,,, in this realm or the next .
this can be **stressful** and **discouraging** for you/~~you~~, for your family, and all the rest . /support
feelings such as ~~fear,~~ ~~impatience,~~ even ~~anger~~ are normal .

while we find you a *gift //you are a gift//* you will meet some of the Psychsocial Team members:

##social workers
##psychiatric nurses
##medical psychiatrists

Your team offers:

~~PP~~education~~PP~~and~~PP~~info~~PP~~before~~PP~~and~~PP~~after
~~pp~~help~~pp~~
~~PP~~counseling~~PP~~emotional~~PP~~support ~~to you and your persons~~

<soul>> it is important you talk to someone. Please <soul>> tell us about your feelings,,,, get *help*,,, listen to your concerts :::: put them in perspective [[[relieving stress can help you feel in control]]]

<_____header>**transplant mentor program<header**

The Transplant Mentor Program can match you or your support person with a post-transplant recipient of *their* support person. The <*gifts*>//mentors are volunteers of the ~~Toronto General Hospital.~~

some ideas of what to talk about:

"what is this experience?"
"what isn't this manual telling me?"
"am I safe?"
"~~god,~~ if I give myself to this, will I, at least, be as strong as I am now after this?"

to be connected to a <*gift*>//mentor,, speak to your transplant coordinator or social worker,, they may refer you… or, contact them directly

~~Toronto General Hospital~~
Transplant Mentor Program
~~416-340-4800 ext. 5655~~
(Psychosocial Reception)

;;reminders while you wait;;

:::driving
if, you have confusion due to your ~~liver disease~~, you **must not** drive.

:::vaccinations
vaccination are important for your health. Please:

—|—flu shot, each and every year
—|—pneumovax vaccine every ~~6~~ years // protects against bacterial pneumonia
—|—hep b vaccine
—|—hep a vaccine

:::dental procedures
please, talk with your doctor or team before any dental work. They may result in bleeding and infection risks //

:::alcohol
to stay on the //wait list\\ for a **liver transplant**, you **must not** drink alcohol.
sorry jewel, but we may do random blood and urine alcohol checks.
if, your tests are **positive**, we <<sorry>> will remove you from the wait list.

please, <soul fused with a body> review the ~~**Alcohol Abstinence Contract**~~ at the front of this manual //you know your hungry ghost// for more info

~~**D.A.R.T. — Drug and Alcohol Registry of Treatment**~~
~~**1-800-565-8603**~~
toll-free, confidential, ~~anonymous~~, 24 hours

:::smoking
quitting,,, smoke is expected.
quitting,,, is an expectation based on health risks,,, including
~~early cardiovascular disease cancer emphysema gastro tract disease~~
quitting,,, may protect you from damaging your heart and lungs.
without,,, *quitting*,,, this damage may make the anaesthesia difficult to manage for this long surgery,, making the recovery slower.

if,,, you ~~quit~~ smoking,,, you will almost immediately see these benefits::::

20 minutes::blood pressure drops
8 hours::carbon monoxide in blood drops
24 hours::risk of having a heart attack drops
48 hours::you will taste and smell more
72 hours::lung capacity increases,,, you will breathe easier
breathe in
breathe out
in
out
in out
2-3 weeks::circulation improves,, so does your lungs
1 to 9 months::fatigue and shortness of breath will decrease
1 year::you cut your risk of heart disease in **half!**

(quit::you have it in you. Smoke free ~~Ontario~~. ~~Government of Ontario~~)

if you need help,,, please ask for help

>>>>traveling / out of town trips <<<<

if,,, <you> are going on a trip, let's talk soon than later… we need to decide if <you> are well enough to travel

if,,, <you> are going out of range for your page,, we need a ## to reach you.
if,,, <you> are traveling out of range and will not be available for transplant, let's talk.
we will put you on {{hold}} on the transplant list

it is your responsibility to notify the transplant office when you return to be reactivated on the waiting list

>>>>over-the-counter medications

for pain,,, take ~~Tylenol(R)~~. Max dose:
4 500mg or ;; 6 325mg

if,,, you need pain medication for more than 2 days, call your family doctor or coordinator
Do **not** take:

#over-the-counter meds
#aspirin or no-steroidal anti-inflammatory drugs like :: ~~Advil |||| Ibuprofen~~
with liver disease, these types of medication may cause bleeding or kidneydamage
#~~Gravol(R) Benadryl~~ .. cold meds .. sleeping pills.. anxiety drugs.. *sleepy*

:::::::**emergency situations**:::::::

<soul, jewel, heart> *if* you/~~you~~ have an emergency, such as ::

((bleeding from stomach or bowels))
((shortness of breath))
((chest/~~heart~~ pain))

call ~~911~~ or fall into your nearest <u>Emergency Department</u>

::::::::::::::::::::::::::::::::multi-organ pre-transplant patient education session::

this could be an important part of you <u>preparing</u> for the journey ahead,, you know,,
transplantation,, and you/~~you~~ are invited to join us.
we welcome you/~~you~~, your family, or support/carry persons to share this experience/prep.
members of the team and a our post-transplant patient ((lived experience experts,,,)) will lead this session and answer any of your questions,,, wonderings,, worries {{no death, no fear}}

topics in this 2 ½ session include ::

:: wait, wait, waited for <u>the</u> ***call***
:: spirit fused with a body|questions {{the real work is done in the privacy of your heart}}
:: social work
:: infection {{this happens to a third,,, sorry}} control
:: the ICU experience {{intensive carry}}
:: the post-transplant journey {{long winding river, step off}}
:: a transplant story // a post-transplant story
{{all we are are stories}}
{{stories all the way down}}
{{telling stories otherwise}}
{{*It matters what stories make worlds, what worlds make stories*}}
{{want a different world, tell a different story}}
{{stories are powerful, they conceal some things and unconceal others}}
{{stories of suffering, stories of bliss,,,,, no mud,, no lotus}}

the pre-transplant patient education session is held multiple times a year
once you slip onto the list, you will be invited.

_____<<vaccines in adult solid organ transplant recipients>>++++